

Structured Clinical Endoscopy Report

Patient Name:	Sarah Jenkins	Patient ID:	PT-3841
Age / Gender:	45 / Female	Date of Procedure:	test_ulcer
Image Quality:	Blur Score: 580	Status:	Validated

Clinical Summary

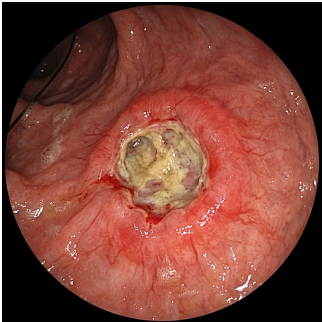
Endoscopic examination of Sarah Jenkins revealed a single gastric mucosal lesion of Mild severity, covering 3.16% of the mucosal field, suggestive of a Gastric Ulcer. High priority follow-up needed.

Endoscopic Findings

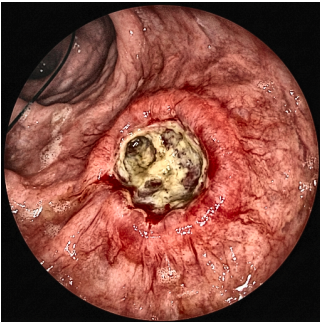
Clean-based or clot-adherent ulcerated lesion observed in the gastric antrum/body. Surrounding mucosa is erythematous. Lesion area is quantified at 3.16% of the field of view.

Condition Identified	Severity Classification	Inference Confidence
Gastric Mucosa Ulceration	Mild	94.0%

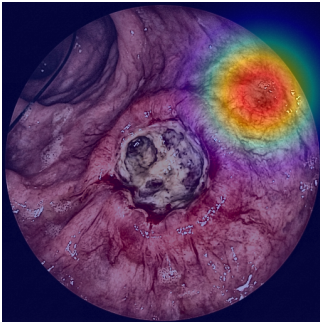
Comparative Multi-Stage Image Overlays



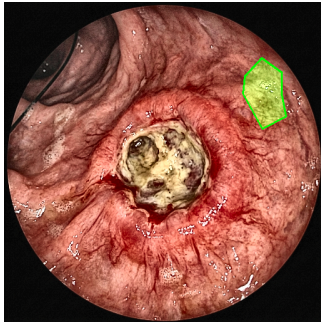
1. Original Frame



2. Enhanced (CLAHE)



3. Grad-CAM



4. Segmentation

Management Plan

- Prescribe Omeprazole 40mg PO daily for 8 weeks (or intravenous PPI if bleeding risk remains)
- Initiate H. pylori eradication therapy (Clarithromycin + Amoxicillin + PPI) if biopsy returns positive
- Strictly avoid NSAIDs, aspirin, and alcohol
- Patient instructed to seek immediate emergency care if hematemesis or melena occurs

Recommended Investigations

- Rapid Urease Test (RUT) or histopathological biopsy for H. pylori detection
- Repeat endoscopy in 6-8 weeks to confirm complete ulcer healing and rule out malignancy
- CBC to rule out chronic blood loss/anemia

Cited Guidelines & Evidence

- ACR Guidelines for Upper Gastrointestinal Bleeding
- Management of peptic ulcer disease and H. pylori infection